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Dear Members of the NITRD NCO,

I am writing to contribute to the ongoing discourse on the development of an Artificial Intelligence (AI) Action Plan, focusing on the transformative potential of AI in the U.S. healthcare system. Our current healthcare infrastructure is marred by inefficiencies that not only inflate costs but also impede the delivery of optimal patient care. AI offers a unique opportunity to address these challenges, fostering a system that is both efficient and patient-centric.

1. Inefficiencies in U.S. Healthcare and the Transformative Potential of AI

The United States spent \$4.9 trillion on healthcare in 2023, accounting for 17.6 percent of GDP. This level of spending is nearly twice as high as that of the next-highest country in the OECD. Despite these expenditures, U.S. healthcare outcomes remain poor compared to peer nations. The United States has the lowest life expectancy and highest mortality rate from preventable and treatable diseases among developed countries. These statistics underscore a fundamental inefficiency: higher spending is failing to translate into better patient outcomes.

A 2019 study published in *JAMA* found that 25 percent of U.S. healthcare spending—between \$760 billion and \$935 billion annually—is waste. The sources of waste are diverse, but a significant portion stems from fraud, abuse of the system, and excessive administrative overhead. These inefficiencies impose direct costs on the healthcare system and also contribute to delays in care, excessive paperwork, and misaligned incentives that prioritize volume over value.

AI offers a transformative opportunity to address these inefficiencies. Multiple studies have shown that AI can enhance fraud detection, flag system abuse, and automate routine clinical and administrative tasks to improve efficiency, accuracy,

and speed in healthcare delivery.^{1 2}AI-driven tools have demonstrated their ability to reduce diagnostic errors, improve hospital operations, and streamline administrative workflows, leading to faster and more cost-effective care.

Despite this potential, AI adoption in healthcare remains extremely limited. Few hospitals have successfully deployed AI at scale, even when evidence supports its benefits. The challenges preventing widespread AI implementation fall into three key categories: operational difficulties in AI development, organizational resistance to AI adoption, and regulatory barriers that limit AI use cases. The following sections outline policy recommendations to address each of these challenges and unlock AI's potential in transforming the U.S. healthcare system.

2. Recommendations

2.1 Overcoming Operational Barriers: Enforcing Healthcare Data Interoperability

Healthcare data in the United States is highly fragmented, often stored in different formats across various providers. This lack of standardization makes it difficult for patients to access their own medical history. A 2021 Pew study found that only 36 percent of Americans reported that all their healthcare providers offered electronic access to their records.³ This fragmentation creates inefficiencies at every level of the healthcare system. Physicians struggle to obtain a complete view of a patient's medical history, delaying diagnoses and treatment decisions. Pharmaceutical companies face challenges in evaluating the long-term effectiveness of medications due to incomplete real-world data. Administrative processes become slower and more complex when providers must manually request and transfer records.

Beyond these immediate problems, the lack of interoperability presents a major barrier to the development and deployment of artificial intelligence in healthcare. AI models rely on large, diverse datasets to generalize effectively, yet the current system prevents models from scaling beyond a single hospital or care network. Without seamless data exchange, AI systems cannot reach their full potential in improving diagnostics, optimizing operations, and personalizing treatments. The federal government should enforce stronger interoperability requirements to ensure that health data can be easily shared across institutions while maintaining

¹ Knight, T., May, B., Tyson, D., McAuley, S., Letzkus, P., & Enright, S. M. (2022). Detecting drug diversion in health-system data using machine learning and advanced analytics. *American Journal of Health-System Pharmacy*, 79(16), 1345–1354.

² Chi, E. A., Chi, G., Tsui, C. T., Jiang, Y., Jarr, K., Kulkarni, C. V., Zhang, M., Long, J., Ng, A. Y., Rajpurkar, P., & Sinha, S. R. (2021). Development and validation of an artificial intelligence system to optimize clinician review of patient records. *JAMA Network Open*, 4(7), e2117391.

³ <https://www.pewtrusts.org/en/research-and-analysis/issue-briefs/2021/07/most-americans-want-to-share-and-access-more-digital-health-data>

security and patient privacy. Concretely, the federal government should mandate one data format (e.g. FHIR) and amend the HIPAA act to drive healthcare data interoperability and improve access for health apps and systems.

2.2 Addressing Organizational Resistance: Financial Incentives for AI Adoption

Many hospitals and care providers are hesitant to adopt AI systems due to high upfront costs, uncertainty about return on investment, and institutional resistance to new technology. To accelerate AI adoption, federal policy should introduce financial incentives for healthcare organizations that implement AI in ways that demonstrably improve efficiency and patient outcomes.

Incentives have repeatedly been one of the most effective drivers of technology adoption in healthcare. A clear precedent is the rapid adoption of Electronic Health Record systems following the passage of the HITECH Act. This law authorized Medicare and Medicaid to provide financial incentives to healthcare providers that adopted and meaningfully used EHR systems. Within a decade, EHR systems became standard across nearly all healthcare organizations.

A similar approach is needed for AI. Federal policy should introduce financial incentives for hospitals and clinics that integrate AI into clinical and operational workflows. These incentives should reward AI systems that enhance diagnostic accuracy, improve patient outcomes, or reduce healthcare costs. As with the HITECH Act, these incentives should transition into penalties for organizations that fail to adopt proven AI technologies within a reasonable timeframe. By tying reimbursement policies to the use of validated AI systems, the government can accelerate the adoption of innovations that improve care delivery.

2.3 Removing Regulatory Barriers: Expanding AI Use Cases in Healthcare

Current regulations severely limit the ability of AI systems to function autonomously in healthcare. No AI system is approved to make independent diagnostic decisions across multiple diseases, and no AI system is allowed to prescribe treatments without physician oversight. While safeguards are necessary to ensure patient safety, these restrictions prevent AI from delivering the efficiency gains it is capable of. If AI-generated recommendations always require a human provider's approval, the benefits of automation remain limited.

Policymakers should focus on creating pathways that allow AI systems to take on greater responsibilities in healthcare, particularly in areas where they have been shown to match or exceed human performance. The approval of autonomous AI-driven diagnosis and treatment should be prioritized, provided these systems meet rigorous safety and efficacy standards. H.R.238 is a strong step in this direction,

proposing to allow FDA-approved AI systems to prescribe drugs autonomously under appropriate regulatory oversight. Similar legislative efforts should be expanded to enable AI to handle a wider range of clinical tasks, reducing the burden on human providers and improving access to timely medical care.

3. Conclusion

By enforcing interoperability, providing financial incentives, and reforming regulations to enable AI autonomy where appropriate, policymakers can create the conditions necessary for AI to transform the healthcare system. These changes will reduce inefficiencies, improve patient outcomes, and allow the United States to remain at the forefront of medical innovation.

Sincerely,

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